



February 23, 2026

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC), Department of Health and Human Services

RE: HHS-ONC-2026-0001-0001, Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Verily welcomes the opportunity to share feedback on strategies HHS can take to accelerate safe and effective AI adoption in clinical care. Recent initiatives established by the Administration, including the OneHHS approach to AI, the HTI-5 rule publication and HTI-2 partial withdrawal, and the introduction of the CMS ACCESS model in conjunction with the FDA TEMPO pilot demonstrate HHS' commitment to advancing the safe and effective use of AI in healthcare delivery. We believe forthcoming policy can and should build on the progress made already by this Administration and the 21st Century Cures Act to enhance access to and exchange of data that will power AI-enabled care solutions and ultimately improve health outcomes for all Americans.

Verily is an Alphabet data platform and AI company that is purpose-built for precision health. We combine deep clinical, regulatory, and technology expertise to provide our customers with patient-centric solutions that make healthcare more personal and precise. Verily solutions are designed to make healthcare data AI-ready and fit for purpose across care, research, and public health. Our extensive experience building innovative technology platforms for data ingestion, curation, and analysis, alongside our commitment to generating high-quality, regulatory-grade real-world data (RWD), positions us to offer perspectives on advancing the collection, standardization, and regulatory acceptance of data derived from real-world sources.

In our response, we highlight three key steps that would help advance the adoption of AI in clinical care: 1) supporting data liquidity within the health technology ecosystem, 2) encouraging efforts to improve data reliability, and 3) establishing national, risk-based frameworks for AI regulation and payment.

1) Fostering a Seamless Environment for Health Information Exchange

Data liquidity—broadly defined as the ease with which health data flows securely and seamlessly across systems, organizations, and individuals—is the foundational element required to accelerate AI adoption. At Verily, we believe it is imperative to build a nationwide and seamless data exchange environment. We acknowledge much progress has been made since the enactment of the 21st Century Cures Act, which provides the authority for The Trusted

Exchange Framework and Common Agreement (TEFCA), a critical tool enabling nationwide data exchange. To increase the usefulness of TEFCA, there is growing momentum among key stakeholders to ensure the consistency of data exchange activities for specific use cases. However, more can and should be done to ensure that patients and providers - and increasingly the AI tools they rely on - have access to complete, accurate data to inform point-of-care decision making. For example, consistent and transparent application of information exchange regulations, standards, services, and policies will help build trust among data exchange partners. Relatedly, increasing the speed of enforcement actions may drive broader participation in the ecosystem.

Additionally, we view the creation of incentives to modernize national data infrastructure and information exchange as a key element to ensuring that AI tools can function safely and effectively in real-world clinical settings. Fundamentally, the speed of data exchange will become an important metric to realize the value of AI tools and their potential to improve patient outcomes at a lower cost. The Fast Healthcare Interoperability Resources (HL7 FHIR) standard can help scale information exchange and access to longitudinal data. FHIR provides the modern, Application Programming Interface (API)-based 'piping' necessary to move data efficiently and securely, however, FHIR implementation remains inconsistent across settings of care delivery. While the HL7 FHIR Roadmap provides a directional north star, implementation may remain slow and fragmented in the absence of incentives to modernize and ensure consistent use of this data standard.

Lastly, for this data infrastructure to be effective, health data should also be discoverable. The development of a provider directory is key for understanding participants in the network. We believe it may be equally important for the network to have a more transparent understanding of data assets available across the exchange environment. Robust metadata would be essential for discoverability and setting standards could help support efficient data exchange activities.

2) Improving the Reliability and Quality of Data to Support Robust and Accurate Insights from AI Tools and Analytics

While fostering more seamless nationwide data exchange is a critical step, improving data reliability is equally important to accelerate the adoption of AI-enabled tools in care delivery. Many AI tools being developed for care rely on data captured in clinical settings, and the fidelity and validity of that data is essential to ensure AI safety and effectiveness. The value of data is determined by how well it can inform and support its intended use. Capturing, curating, and linking data that meets regulatory standards is a process that depends on an ecosystem of stakeholders. The creation of incentives, including those that are financial and reimbursement-related, will help improve both data exchange and reliability.

We strongly urge the Assistant Secretary for Technology Policy (ASTP/ONC) to maintain and expand its leadership in national efforts to improve data quality. The United States Core Data for Interoperability (USCDI) framework provides the essential technical and policy foundation required for interoperability by establishing a consistent 'floor' for data expectations. The USCDI seeks to eliminate the ambiguity that has historically hindered information sharing, but more work is needed to push a common understanding of what “fit for purpose” data looks like in a given context. For medical product development, the FDA has advanced thinking around use of real-world data (RWD) and real-world evidence (RWE), outlining what may qualify as regulatory-grade data. FDA’s considerations for data relevance and reliability could be more firmly aligned with ASTP/ONC’s interoperability standards as well as with CMS’ innovative payment models, enabling HHS to effectively build a robust health technology ecosystem that encourages adoption of AI care tools.

3) Establishing National Risk-based Frameworks to Guide Regulation and Payment

We urge HHS to provide national leadership on standards and policies for AI development and deployment in the healthcare system. A state by state approach will only fragment and slow innovation as organizations struggle to implement varying sets of requirements, validation methods, and regulations. A unified approach to performance evaluation, ongoing safety monitoring, and payment will be essential for building trust in transformative AI tools.

To accelerate adoption of AI care tools, HHS’ development of an effective regulatory framework that collectively considers an AI tool’s approval, post-market monitoring, and reimbursement is critical. An effective regulatory framework for AI should be developed in partnership between CMS, the FDA, ASTP/ONC, other relevant agencies and external stakeholders (e.g., professional societies, accreditation non-profits). The recently introduced CMS ACCESS payment model and FDA TEMPO pilot provide an initial example in chronic care management for how such a framework that integrates evaluation, monitoring, and payment could be implemented.

We suggest that the approach to evaluation and real-world monitoring for AI-enabled care tools be titrated to the level of risk exposed to the patient or consumer, building on how risk is classified for SaMD.¹ This could mean that a lighter regulatory touch, if necessary, is acceptable for lower-risk AI tools, whereas higher-risk AI tools might require more continuous and comprehensive data capture, mandatory reporting, or an expert in the loop to ensure ongoing safety and effectiveness.

Payment models could align with a risk-based regulatory approach by incentivizing adoption of AI care tools that are appropriately monitored according to their classified product risk level.

¹ <https://www.fda.gov/medical-devices/software-medical-device-samd/global-approach-software-medical-device>



Interagency harmonization of real-world performance monitoring across regulatory approval requirements and payment structures could not only help regulators keep pace with the rapid speed of innovation, but it could also improve efficiencies when implementing statutorily-defined authorities across approval and payment. Additionally, this coordination would provide more predictability and instill greater confidence so that AI developers continue to invest and innovate.

Closing

Verily believes that appropriate, safe, and effective deployment of AI in care has enormous potential to prevent and better manage disease, and accordingly, we are building technologies that can effectively integrate multimodal health data sources. AI is already transforming how care is delivered, and as AI continues to evolve, the importance of HHS' leadership in this area will only continue to grow.

Thank you for the opportunity to comment on these timely and critical topics. We applaud HHS' commitment to ensuring accessible, high-quality care for all Americans, and its focus on accelerating adoption of AI-enabled care. We look forward to partnering with the Administration and other stakeholders to develop and implement innovative AI solutions that make this vision a reality.

Sincerely,

Laura O'Donnell

Laura O'Donnell
Chief Legal Officer and Chief Policy Officer
Verily Life Sciences, LLC